

COURT FILE NO.: 03-CV-253544CM1

DATE: 2008/06/05

**ONTARIO
SUPERIOR COURT OF JUSTICE**

Between:

Ju-Kyung Song, Gyu-Yub Song, Jung-Suk Park and Min-Ji Song

Plaintiffs

And:

**Ji Wan Hong, Enterprise Rent-A-Car Canada Limited, and Allianz
Insurance Company of Canada, Jong Min Lee and Joo Yong Park**

Defendants

Appearances:

Counsel for Plaintiffs:	John A. McLeish and Meighan Ferris-Miles
Counsel for Defendants:	Barry A. Percival, Q.C.

DATE HEARD: On Written Submissions

Endorsement

[1] Ordinarily, the court fixes costs of an action after trial, at a time when all of the evidence tendered by the parties has been heard and considered. In this case, however, the action settled during trial with the result that the evidence to be heard from six expert witnesses and, possibly, from other witnesses has not been heard.

[2] By way of an endorsement made on 10 March 2008 at a mid-trial pre-trial hearing, Wilkins J. confirmed that the action had settled for \$3,000,000.00 and ordered that costs of the action be determined by the trial judge. I will do what I can in the circumstances to assist the parties to bring closure to the settlement.

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The Circumstances

[3] This action arises from a motor vehicle accident that occurred in July of 2002 near Cornwall, Ontario. The plaintiff, Ju-Kyung Song ("Eugene"), was a passenger in a motor vehicle involved in the accident and, as the result of the vehicle rolling over and, apparently because Eugene was not secured within the vehicle by a seat belt, she was ejected from the vehicle and suffered a variety of injuries which have left her an incomplete paraplegic and catastrophically impaired.

[4] Eugene was a university student in her home country, Korea, and was visiting Canada during her vacation when the accident occurred. She was treated initially in hospital in Ottawa and then returned home to Korea for additional medical care and rehabilitation, both in hospital as an inpatient and, afterwards, as an outpatient.

[5] She continues to access therapy treatments, albeit on a much reduced basis and, for the time being, she is gainfully employed on a full time basis in a clerical, administrative job in Seoul, Korea working for KAL, Korean Airlines.

[6] The evidence heard to date, during 10 days of trial from 10 witnesses and through 37 exhibits, including extensive hospital and health care records from hospitals in Canada and in Korea, clearly establishes that Eugene has been treated as effectively as is possible to restore some of the sensory and motor

functions that were compromised by spinal cord injuries sustained in the accident.

[7] The parties agree that the constellation of symptoms and complaints that Eugene has been left with as the result of her spinal cord and her other injuries are serious and permanent. There is no suggestion in the evidence that Eugene could enjoy any meaningful improvement in her functioning in future and the evidence heard to this point clearly demonstrates that nature will not be kind to Eugene as she ages. She is clearly at risk of experiencing degeneration in her functioning which in turn will adversely affect activities of daily living as well as producing degeneration in her muscle strength and stamina. Her skin is now and will become increasingly susceptible to breakdown.

[8] Eugene and other witnesses described the challenges that she faces with activities of daily living, from simple things such as brushing her hair to rolling over in bed. There is very little in the way of normalcy in Eugene's life as a 26 year old, wheel chair dependant, single woman as compared to the lives led by other people of her age.

[9] Eugene experiences partial to total loss of motor and sensory functions in all four limbs. The muscles of her abdomen and trunk are compromised, as is the functioning of her bowel and bladder.

[10] Although she can stand and walk for short distances with support, her balance is impaired and she is at risk of falling. Indeed, the evidence has shown that Eugene has fallen often and, on one recent occasion, late in 2007, a fall produced a fracture injury to one of her ankles.

[11] The above is by no means a complete summary of Eugene's abilities and limitations but it is an appetizer that will suggest the flavour of the evidence heard on damages issues. For a more generous serving of information surrounding and supporting the medical evidence, see the attached summary of the testimony heard from the health care experts heard at trial. This is taken from my notes but is organized in the form and content likely to approximate that which I would have incorporated into my charge to the jury at the conclusion of the trial.

[12] In a case such as this one, a case in which the jury would necessarily consider complex medical issues and evidence, especially as there would be assertions made by the defendants that the available evidence does not support future income loss claims and future care needs and costs as submitted by the plaintiffs, I would be inclined to be more inclusive than selective in my review of medical evidence for the benefit of the jury.

[13] It must be said that neither plaintiffs nor defendants sought an order to strike the jury during this trial. In light of the ruling I released on 10 March, last, there may have been objections made to the qualification of three of the six expert witnesses scheduled to be called, had the trial continued and run its course. There may have been objections to the hearsay nature of some of the evidence expected to be heard from some or all of the remaining witnesses. Nevertheless, there is no basis for me to conclude that the case would not ultimately have been turned over to the jury.

[14] Liability for the accident and contributory negligence for failure to wear a seat belt were resolved prior to trial. The jury was made aware of these facts at the outset and the issue remaining for jury determination was therefore the issue of damages.

[15] Mr. McLeish told the jury in the plaintiffs' opening statement that In addition to the spinal cord injury, this plaintiff suffered fractures at C6, C7, T4, plus fractures to her jaw, both upper arms near her shoulders and her pelvis. As well, she had an injury to her spleen which required a splenectomy and she suffered a brain injury. The evidence led thereafter bore out his predictions.

[16] He concluded by asserting that among the questions that the jury would be given at the end of the case will be a question calling for a determination of the plaintiff's income loss to the time of trial and her future income loss and, as well, expenses that she will face in the future including housing, cost of wheelchairs, cost of caregivers and medical expenses.

[17] Mr. Percival opened to the jury with a review of some of Eugene's rehabilitation accomplishments: she did manage to complete her university degree and she is able to operate a motor vehicle, for example. He admitted in his opening and throughout the trial, however, that there is no question but that Eugene suffered serious injuries.

[18] The particulars of the settlement of the damages issues in this case are not before the court on this application. Whether or to what extent the settlement includes components of Eugene's general damages claims for pain and suffering and loss of amenities of life, her parents' and her sister's claims for loss of care, guidance and companionship, Eugene's claims for loss of past or future income earning ability and her claims for future losses, under heads of damages identified in the pleadings such as housekeeping, medication, therapy, rehabilitation, attendant care, specialized equipment, modified housing, transportation and other forms of care, are not before me.

[19] The pleadings detail a claim for loss or compromise of any opportunity Eugene may have had to enter into a common law relationship or to marry. I have no basis to know whether that claim is compensated by the settlement.

[20] Although pleaded, at trial the plaintiffs abandoned claims by family members for special damages arising from travel between Korea and Canada.

[21] Whether or to what extent the settlement reflects any reduction of the plaintiffs' claims due to the fact that accident benefits claims were paid to family members for attendant care services that Eugene's parents have provided is not known.

[22] Whether the settlement agreement evidenced by trial exhibit 14, Eugene's settlement of past and future accident benefit claims as at 23 January 2006 in the total amount of \$1,025,000 factored into the settlement of the damages issues that otherwise would be determined at this trial is also unknown.

[23] Notwithstanding the fact that the anticipated evidence at trial was not fully led and that the particulars of the settlement are largely unknown, this court is asked to assist the parties to a determination of fee and disbursement items of costs to be paid by the defendants to the plaintiffs.

Analysis

[24] The parties understand and agree that the purpose of the exercise is to fix and not to assess costs.

The Positions of the Parties

The Plaintiffs' Position

[25] For reasons detailed in the Costs Outline of the Plaintiffs (Form 57B), the plaintiffs seek an award of costs to be fixed as follows:

Fees (detailed in the <i>Bill of Costs</i>):	\$349,990.20
Estimated counsel fee for appearance:	\$2,500.00
Disbursements (detailed in the <i>Bill of Costs</i>):	\$416,663.63
Total:	\$769,153.83

[26] *Inter alia*, the Plaintiffs submit that the fees sought are reasonable in light of the time necessarily expended by the lawyers and other timekeepers who represented the plaintiffs in connection with the preparation for and conduct of the trial of this action, including all associated procedures and the negotiation of the settlement.

[27] The plaintiffs submit that:

The Court must decide if the hours spent are reasonable. In *Carpenter v. Malcolm*, Mr. Justice Catzman, as he then was, held that counsel was entitled to be paid for the full amount of docketed time. He said in his reasons that counsel are entitled to discharge their obligations without expecting an arbitrary reduction.

If counsel are to discharge the functions which the Courts expect them to discharge, and on occasion fault them for not discharging, they ought to be able, in my view, to expect that their clients' party-and-party costs will be assessed in a manner that reasonably and without arbitrary diminution acknowledges the effects legitimately expended in that connection. (*Carpenter v. Malcolm*, [1985]O.J.No.1889(H.C.J.))

See also: *Roberts. v. Morana*, *supra*, at para 7, which cites *Carpenter v. Malcom* with approval.

[28] They add that:

[T]he function of the Court is not to assess the amount of time with hindsight, but rather to determine if any of the time claimed is so grossly excessive as to be obvious overkill. In *Tri-S Investments v. Vong*, Madam Justice Feldman said the following:

I do not view it to be the courts function when fixing costs to second guess successful counsel on the amount of time that should or could have been spent to achieve the same result, unless the time spent is so grossly excessive as to be overkill. (*Tri-S Investments v. Vong*, [1991] O.J. No. 2292 (Gen. Div)).

[29] With these authorities, I fully concur

[30] The plaintiffs point to the training and experience of the timekeepers and submit that the rates that each of them charged is reasonable. They refer to my reasons in *Marcoccia (Litigation guardian of) v. Ford Credit Canada Ltd.*, [2007] O.J. No. 4656 (S.C.J.) at paras80 and 81, in which I describe some of the difficulties invariably faced by counsel in the conduct of a complicated case at trial.

[31] I have every confidence that my concerns identified in the referenced passages in *Marcoccia (supra)* apply to the instant case as well. By implication, the plaintiffs seem to be saying that the application of rates at the upper end of any reasonable scale of rates to be applied to the timekeepers in the instant case and on a partial indemnity basis is warranted, at least in respect of time charged for the preparation for and attendance at the trial itself, the time at issue in *Marcoccia (supra)*.

[32] The plaintiffs submit that it was reasonable, in the circumstances of a case of this size and complexity to have Mr. McLeish assisted by Ms. Ferris-Miles. Reasons are enumerated as follows:

1.

- a.) To take accurate notes, especially when witnesses were being examined in chief by Mr. McLeish.
- b.) To keep track of the documents and items made exhibits at trial.
- c.) To permit counsel to fluidly present demonstrative evidence to the jury without interrupting or delaying the examination of witnesses or opening submissions.

- d.) To ensure the coordination of witness scheduling.
- e.) To assist with preparing witnesses for trial testimony. It was not feasible for one counsel acting alone to prepare each of the witnesses. Ms. Ferris-Miles assisted Mr. McLeish by preparing the Plaintiffs for trial. Ms. Mazzucco assisted Mr. McLeish by preparing three lay witnesses that the Plaintiffs intended to call to give evidence by audio-video conference.
- f.) To share examination of the witnesses. Ms. Ferris-Miles assisted Mr. McLeish in this regard by conducting the examination in chief of three of the Plaintiffs.
- g.) To permit counsel to respond quickly to issues that arose during the course of trial, which necessitated legal argument. Mr. Rikin Morzaria, an associate at McLeish Orlando LLP, researched issues and prepared submissions for the Court on issues arising from tactical decisions made by defence counsel during the course of trial. Mr. Morzaria's involvement allowed the trial to continue, with minimal delay to deal with issues that arose after the trial started.

[33] The Plaintiffs submit that when considering the proportionality of fees between the unsuccessful party and the winning party, one should also consider the following factors:

- Whether the complexity involved in preparing both parties' witnesses is comparable.
- Whether both parties have prepared and called their witnesses in support of their case.

[34] The plaintiffs submit that the fact that the parties and many witnesses live in Korea and speak little if any English created practical complexities in preparing for the necessary presentation of their evidence at trial.

[35] Further, the plaintiffs submit that the present situation is unusual in that the trial judge is being asked to fix costs where the case settled before the conclusion of the trial. The plaintiffs called ten witnesses over the course of two weeks. They had prepared their four remaining witnesses to give evidence. Conversely, the defendants had not started to call their evidence at the time of settlement. It would, therefore, be expected that the plaintiffs' costs would be higher than the defendants' at this point in time.

[36] As to disbursements, the plaintiffs submit that each item of disbursements identified is supported by the referenced invoices and that each represents cost incurred or paid by counsel and/or the plaintiffs in order to fully prepare and present the plaintiffs' case at trial, in accordance with the spirit and letter of the law.

[37] They assert that the experts retained "had to read voluminous documentation before trial, including hospital records and medical reports".

[38] As the case settled before the plaintiffs' case closed, several of the experts whose accounts are identified in the plaintiffs' disbursements list did not attend at trial and did not, therefore give their evidence to the court. Nevertheless, the plaintiffs assert that "The costs of witnesses who have been summonsed, but ultimately not called to testify is an appropriate expenditure involved in trial preparation, and should be allowed. *Ishoy v. Abreu*, [2005] O.J. No. 5702, at para.13.

[39] Certain of the disbursements claimed cannot be ascribed to any particular item in the Tariff; the plaintiffs rely upon the reasoning of Lane J. in respect of such items and point out that the court has the discretion to allow such items to be fixed and allowed as disbursements, as Lane J. did in *Banihashem-Bankhtari et al. v. Axes Investments Inc. et al.* (2003), 66 O.R. (3d) 284 (S.C.J.) and as the Court of Appeal for Ontario did in *Moon v. Sher*, [2004] O.J. No. 4651 (C.A.).

[40] The plaintiffs submit that the costs claimed must involve amounts that the defendants could reasonably be expected to pay.

[41] They add that the costs sought must be measured as reasonable in relation to the amount in dispute in the case. They quote and rely upon cases so saying; for example:

If the plaintiff had been unsuccessful [the amount of the judgment] would not have been recovered. The plaintiff was entitled to incur legal expenses commensurate with the amount in issue (emphasis added).

Tri-Associates Insurance Agency v. Douglas (1986), 9 C.P.C. (2d) 227 (Ont H.C.J.) per Rosenberg J., at 228-29.

See also: *Moon v. Sher*, [2004] O.J. No. 4651 (Ont. C.A.) at para.31.

[42] The submission here made is that the instant case involved large exposures to damages for the defendants. Even after deducting 20% for contributory negligence on account of the seat belt issue and after providing the defendants a credit for accident benefits recovered before trial, a settlement of \$3,000,000.00 was achieved.

[43] It is asserted that the case involved many and complex issues, both factual and legal. I agree.

[44] The plaintiffs list several steps taken to minimize the length of the trial and suggest that these should weigh favourably in the balance in the process of fixing costs in this case.

The Position of the Defendants

[45] The principal position asserted by the defendants is that there was excessive, over preparation of the plaintiff's case. In addition, the defendants question the accuracy of some of the docket entries pointing two occasions of a parent duplication of dockets.

[46] The defendants apply words such as "inordinate and unreasonable in all of the circumstances" to their description of the time docketed by plaintiff's counsel. They Ed that "this was simply a case of gross over-preparation and over-lawyering".

[47] The defendants maintain that the amount being claimed for costs and disbursements is grossly excessive and results in an amount claimed for costs that is neither fair nor reasonable.

[48] In connection with the matter of be reasonable expectations of the defendants, it is submitted that a comparison of the number of hours docketed by defense counsel and by plaintiff's counsel confirms the contention that plaintiff's counsel spent too much time in preparation for this case.

[49] The defendants note that liability and contributory negligence issues were settled before trial and that the action proceeded as an assessment of

damages only. Further, the defendants assert that the "only unusual issues at trial were a) the significance of a settlement of the SABs claim in January 2006 for approximately \$1 million, and b) the proof of anticipated future care costs in Korea".

[50] The defendants refer specifically to various items of disbursements. For reasons detailed in their submissions, the defendants submit that these items it should be reduced or, in some cases, eliminated. Among the reasons given is the assertion that the disbursement does not represent the cost of evidence at trial.

[51] As well, the defendants point to the Tariff and assert that where disbursement items do not track the provisions of the Tariff, they ought not be allowed.

[52] The defendants concede that reasonable amounts should be awarded in respect of disbursements incurred for travel expenses and photocopies, for example, but assert that the disbursements sought are excessive. In respect of photocopies, the defendants submit that the amount claimed is "so excessive to be truly suspect".

[53] In concluding paragraphs, the defendants restate and overall concern that the amounts claimed for fees and disbursements are too high. The defendants urge the court to "conclude that the docket entries are inflated, or if not inflated, represent unreasonable, inordinate and excessive "legal churning" in this matter".

[54] The defendants submit that "the amount being claimed for partial indemnity costs by the plaintiffs is not fair and reasonable in all of the circumstances of this case, and far beyond the reasonable expectation of the defendants."

Analysis

[55] In fixing costs, the court need not attempt a line by line analysis of the bill of costs under consideration but nor should the process become a mathematical exercise of applying hourly rates to docketed hours. There must be a balance achieved between recovery of a fair and reasonable amount for services rendered and disbursements incurred considered in the context of the particular case and the reasonable expectations of the party called upon to pay the amount to be fixed. Put another way, the court is ever mindful of an "overriding principle" of what is a fair and reasonable amount to be paid in costs

and this is not driven by the cost or expense of litigation incurred by the successful party.

[56] The starting point for the process of fixing costs following the completion of a given trial is Section 131 of the *Courts of Justice Act*, R.S.O. 1990, c.C.43 and the general principles outlined in Rule 57 of the Rules of Civil Procedure, R.R.O., Reg.194.

[57] The rule addresses the process for fixing costs at the adjudicated conclusion of a matter and requires the court to devise and adopt the simplest, least expensive and most expeditious process for fixing costs {Rule 57 (7)}.

[58] Rule 57 does not refer to situations where, as here, the parties settle the action during the course of a trial and request that the court sort out for them their differences on costs issues.

[59] As indicated above, this is an unusual and unfortunate situation to place a trial judge into. The parties have the advantage of having lived with the issues in the case for many years whereas the court knows no more about the issues than the evidence at trial and the opposing submissions from counsel disclose.

[60] The presiding judge at the pre and mid-trial settlement conferences may have developed an understanding of the issues resolved and those remaining and may have been better positioned to understand the strengths and weaknesses of the facts that have not yet but might become evidence if the trial had continued.

[61] The defendants' submissions reveal that case settled for a greater amount than the defendants had offered prior to trial. Perhaps the evidence at trial encouraged the defendants to increase their offer or perhaps evidence anticipated but not yet led at trial encouraged the increase.

[62] As I was not privy to settlement negotiations and as the trial did not run its course to jury verdict and final disposition reflected by an endorsement on the Trial Record, I cannot rigidly apply Rule 57 to produce a fair result in fixing costs here.

[63] In this case, and on consent, the court invited brief written/electronic submissions to be delivered within 1 week of the settlement. Neither the timeline nor the brevity of submissions requirements were followed.

[64] Factors that the court ordinarily considers in arriving at fair figures for fee items of costs include the experience of the lawyers for the parties entitled to costs as well as the rates charged and the hours spent by those lawyers. These factors are features of a consideration of the time and the nature of the efforts

required to properly prepare and present a case. (*Moon v. Sher*, [2004] O.J. No. 4651 (C.A.) at para.35)

[65] In this case, the plaintiffs were represented principally by Mr. McLeish, an accomplished, skilled and experienced trial lawyer and as well, the plaintiffs were represented by Ms. Ferris-Miles, an associate counsel who demonstrated skill and ability in addition to her obvious preparation for and understanding of the issues and evidence at trial.

[66] The court may also consider the amount of costs that the parties paying costs could reasonably expect to pay in relation to the costs being fixed. In this regard, the court may refer to factors such as:

- the amounts claimed and recovered;
- the importance of the issues;
- and the complexity of the proceeding;

[67] For the reasons set out above and because the court did not have the benefit of hearing the six witnesses in question and the closing arguments of counsel, it is difficult to predict the amounts of money that the plaintiffs would submit the jury should award for all or any of the heads of damages to be determined. That the plaintiffs recovered the sum of \$3,000,000.00 in settlement of their claims, however, suggests a measure of success that cannot lightly be ignored.

[68] On the other hand, this court has no way of knowing what motivated the plaintiffs to settle their claims, let alone for the figure agreed to. The settlement resolved all claims of all four plaintiffs for damages and interest (the allocation of settlement components among the plaintiffs has not been disclosed) but the quantum of this settlement is a far less accurate measure of success for any of the four plaintiffs than a jury verdict would be.

[69] The issues to be determined in this case were of enormous importance to the plaintiffs. Eugene's future financial security rode in the balance. Her ability to live independently with a caregiver to assist with her activities of daily living was a very real concern to every member of Eugene's family.

[70] Eugene's parents and, to an extent, her sister too, gave unstintingly of their time and attention over the years since the accident, providing Eugene with necessary, constant and compassionate care and attention. They slept on floors in order that Eugene might enjoy the comfort of a bed in the small bachelor

apartment that they occupied together. They cooked, cleaned, washed and helped with hygiene, transportation and school needs.

[71] The issues in this case were also complex, this because the nature and number of Eugene's injuries and the constellation of her complaints and limitations were complex and many were inter-related. Lay witnesses and experts alike spoke of signs and symptoms of fatigue, inability to concentrate, memory challenges and other features of Eugene's overall situation that may be related to physical injuries, to emotional functioning difficulties or to both. Clearly the rehabilitation process as well as Eugene's ability to undertake and sustain activities of normal daily living have been and will continue to be complex and constant concerns.

[72] This was by no means a simple assessment of damages case.

[73] This action proceeded to trial with all damages issues outstanding. The result was a long, complicated and very demanding trial for the parties and for their counsel. This said, the trial was thoroughly planned out and presented. Trial time was time well spent. All aspects of Eugene's injuries, her treatment for and recuperation from her injuries, her past hopes and present goals for the future, her functioning in her home, social and work environments, her limitations, physical and psychological, were comprehensively addressed in a clear and concise fashion for the benefit of the jury by the many witnesses heard from.

[74] As all four plaintiffs spoke very little English, their evidence at trial was, of necessity, heard through Korean interpreters. This fact added an additional layer of complexity to the proceedings and a very real challenge to counsel to convey the meaning and subtleties of concepts, questions and answers, both because of and despite the translation process. This fact also speaks to the preparation and advocacy skills demonstrated by plaintiffs' counsel, indeed by all counsel, who participated in this trial.

[75] While it is not unusual in cases of this size and complexity that the number and bulk of exhibits be substantial, in this case the exhibits were uniformly well organized and presented, often with innovative and helpful illustrations or photographs and almost always with index documents for each binder. The result is a body of exhibits that is rather less daunting than it would otherwise be and enormously more user-friendly for the jury.

Ruling on Expert Evidence

[76] As noted above, the court released a ruling on the very day that the parties concluded their settlement discussions and sought the endorsement that Wilkins J. authored and that brought this application before me.

[77] Had the trial continued, the upshot of the ruling may have been that the jury may well not have heard some of the opinions contained within the reports written by the three experts referred to in the ruling, Ms. Baptiste, Mr. Baum and Dr. Coyte,. As the ruling makes clear, the evidence of these three experts addresses Eugene's future care needs and costs claims.

[78] To the extent that the evidence of these three witnesses would not directly reach the ears of the jurors, it might be that the likelihood of success for the plaintiffs would suffer. On the other hand, it must be remembered that while the onus is on the plaintiff to establish claims for damages, including claims for losses or damages in the future, the burden of proof differs depending upon whether damages are experienced in the past and present or whether they fall to be experienced in the future.

[79] As to future claims, the jury in this case would have been instructed that the law requires proof upon a standard lower than proof beyond a reasonable doubt, perhaps even below the level of reasonable probability. The jury in this case would have been instructed, had the case not settled, that:

The onus is to demonstrate a reasonable chance that a loss or expense will arise in the future. Put another way, the plaintiffs must show a real and substantial possibility of future loss.

If the plaintiffs establish that a real and substantial risk of a future care cost, rehabilitation cost or replacement of household services or other expenses will arise, they are entitled to compensation for that cost based upon the degree of possibility or percentage of risk that you find that this future loss will arise. Compensation for future losses is not an all or nothing proposition.

The greater the risk of loss, the greater will be the compensation.

[80] The jury would be charged that the quantum of future care and other future loss claims would be established by application of a two step process. First, if the jury decides that Eugene will need attendant care, for example, throughout her life on the basis of a real and substantial risk of this loss occurring in the future, then the entitlement to an award for this component of future care is made out.

[81] The second step in the process requires that the jury consider the evidence it has heard or that it sees in the exhibits on the cost of providing, in this example, attendant care services over the period of time the jury considers to be appropriate.

[82] The jury would be instructed on the difficulty involved in predicting the future and upon its right and duty to consider contingencies, both positive and negative.

[83] The jury would undoubtedly have evidence from the accounting expert(s) about the present value of \$1000.00 calculated over Eugene's life expectancy and, similarly, the present value of each \$1000.00 of income lost annually over her working life expectancy.

[84] The point is that some evidence might well trickle in through the examinations and cross-examinations of the three witnesses subject to the court's ruling. In addition, the jury would be at liberty to and would be required to consider all the evidence from all sources on future claims.

[85] The jury might consider the evidence of Dr. Tator and recall that he was asked, without objection from Mr. Percival, for his views upon Eugene's future care needs and the contents of the reports of Ms. Baptiste. He replied that he had read and agrees with her opinions. Specifically, he stated that because Eugene has multiple disabilities due to three major injuries to her neck, thoracic region and brain and because it is unusual to have three anatomical regions damaged in the nervous system, he feels that she does need almost constant attendant care.

[86] For one thing, Dr. Tator opined, she has memory difficulties including difficulty in recognition of danger. She would require someone to be with her because of what he called cognitive, cerebral factors.

[87] He added that the fact of the existence of two other additional levels of injury means that virtually every activity of daily living is impaired significantly. For example, he said, even simple acts like taking off boots are impaired because Eugene does not have coordination and the strength sufficient to do that and therefore, she needs help. Although she can walk and stand to some degree, because of balance and weakness problems and perhaps because of judgment problems as well, she is at risk. She recently fell and fractured her ankle and that, he said, is "a wonderful example of why she needs people with her".

[88] The jury may recall that Dr. Ditor, a kiniseologist, told them that Eugene would benefit from the availability and use of exercise equipment and support staff to use that equipment safely and for her maximum benefit in rehabilitation. He supplied costs of certain equipment that he considered to be appropriate.

[89] Cathy Butler, a physiotherapist, gave evidence about future challenges Eugene will face at home, at work and out in the community. She felt that Eugene has reached her maximum medical recovery. Eugene presents with mobility, balance and strength issues that would prevent independent self-care. She needs housekeeping assistance even if she lives in a wheelchair accessible home. She is restricted by her endurance problems and can not self propel over distances in her wheelchair and will have difficulty transferring from her wheelchair into cars.

[90] She felt the Eugene would have difficulty finding employment or working at a work site because she is wheelchair dependent and has low finger dexterity speeds. Overall, Eugene will need a benevolent employer because of her limitations, Ms. Butler felt. She did not feel that Eugene could find and keep competitive employment.

[91] If Eugene were to have children, she would need help with raising them. Ms. Butler felt that Eugene would require an adaptable home and vehicle.

[92] Mihwa Choi, an occupational therapist, gave evidence at this trial. She formed an opinion that Eugene requires attendant care on a 24 hour per day basis because the strength and range of movement in her upper extremities is impaired and Eugene is also at risk of falling due to balance and strength issues.

[93] Ms Choi was concerned that Eugene may not react appropriately to emergency situations. Although Eugene is capable of self propulsion in her wheelchair over short times and distances, in Korea the terrain is hilly, bumpy and uneven and Eugene needs help there and to transfer from her wheelchair to a car.

[94] Ms. Choi spoke of seeing a book containing official Korean health care costs for services and treatments; she sourced other care and services costs that she provided in her evidence to the jury and that she was cross examined upon. The jury could form its conclusions both upon the evidence Ms. Choi gave regarding future care needs and as to the associated costs.

[95] By no means are the needs and costs references stated here meant to be exhaustive; rather they are illustrative of the fact that there was some evidence upon which the jury could consider the future care and future income loss claims, regardless of what the jury may (or may not) have heard from the three expert witnesses subject to the ruling and/or from the remaining three witnesses scheduled to follow them.

[96] Further, it may be that the test for hearing the three witnesses subject to the ruling upon all or any of the evidence containing second hand or hearsay

information taken in Korea from people who would not have attended as witnesses at trial should involve the application of a low bar. Given that the evidence is aimed at establishing only a substantial possibility and not a probability of a loss arising in the future, arguably a lower standard should apply to accepting second hand information as evidence, provided of course that such information is accepted as being reasonably reliable evidence.

[97] Further, to the extent that Dr. Tator has endorsed opinions set forth in the Baptiste reports, at least to the extent that those opinions engage Dr. Tator's area of expertise, arguably those opinions have been established and have full evidentiary value for the jury.

[98] I declined to predict for counsel during the trial and prior to the settlement of the action just what impact my ruling might have on the process by which the affected three expert witnesses might give their evidence. I decline to predict it now. Likewise, I decline to accept that the jury could not have found evidence upon which to establish future claims awards. The task may well have been challenging but I have every confidence that this jury would have given its best effort.

Additional Considerations

[99] In my view the defendants faced a real and present danger, at the time this case settled, that if the trial had continued, substantial awards of damages may have resulted. In these circumstances, I am not prepared to agree with the defendants and to find that any of the time spent by plaintiffs' counsel was not necessary and appropriate.

[100] That defence counsel shared with the court particulars of the time docketed in connection with meeting the plaintiffs' claims throughout the course of the action is both welcome and helpful but it is by no means determinative of the issue of what the defendants' expectations should be with respect to the plaintiffs' costs claims.

[101] As well, I accept the plaintiffs' explanation that more time and effort was required to prepare and present the plaintiffs' claims than to defend them.

[102] Whether by design or oversight, I note that the defendants are critical of time spent and disbursements incurred but have offered no assistance in determining what numbers the defendants accept as reasonable and no description of the defendants' expectations on costs issues.

[103] I am not so far away from my time at the Bar that I have forgotten the expectation of defendants who settle personal injury cases for substantial amounts in damages. Defendants generally expect to pay costs of between 10% and 15% of the damages paid in settlement plus disbursements and GST. Given the nature and complexity of this case and the fact that it settled at trial, I would be very surprised to learn that the defendants expected to pay costs of less than 10% to 15% in this case. Unfortunately, however, the defendants have chosen to keep their expectations to themselves.

[104] As the particulars of the settlement are not before the court, I cannot accurately compare damages (exclusive of interest) to costs. I note however that the fee portions of costs claimed appear to be in the range of 10% to 15% of damages, as future claims would not attract pre-judgment interest and most of the damages paid likely represent resolution of future claims.

[105] It was entirely appropriate for the plaintiffs to have had senior and junior counsel briefed and ready to start the trial. It was also appropriate that junior counsel participated in leading evidence from witnesses at trial. Having junior counsel present as a note taker while senior counsel led evidence was an expensive luxury that the defendants need not pay.

Disbursements

[106] Turning now to disbursements, I am not prepared to reduce the amounts claimed because witnesses were not called or demonstrative aids were not introduced into evidence at trial. I am not in a position to gauge the impact of such evidence on the negotiation and settlement process. As noted above, the defendants increased their offer and I cannot conclude that only the evidence led at trial influenced their decision making process.

[107] I agree with the defendants that they should not be ordered to pay for Mr. Dick's travel expenses (Bill of Costs, Tab 1, page 7) and order that the sum of \$7,274.50 be deducted from the disbursements claim.

[108] I am troubled by the disbursement claimed for the services rendered by Ms. Baptiste. In light of my ruling, it may have developed that her evidence might not have tracked the contents of her reports. I will reduce the disbursement claim in her case by approximately ½ the amount claimed in Items 62-65 (Tab C, page 3/8) for a reduction of \$14,000.00.

[109] Likewise, Items 67-68 (Tab C, page 3/8) should be reduced by \$7500.00.

[110] The defendants assert that the Medical Legal Video (Item 103-108) was not produced. As such, I cannot conclude that the expenses incurred in relation to the preparation of the video influenced the defendants in their settlement deliberations. The video was not shown at trial and it is unlikely that it would have been, as it had not been produced. The disbursements claim will be reduced by \$23,598.27.

[111] I accept that the defendants should not be required to pay for travel expenses incurred in connection with retaining counsel in Canada. The disbursements claimed in Item 109 (Tab C, pages 5/8 and 6/8) are disallowed. The disbursements claim is therefore reduced by a further \$17,800.

[112] The defendants should not be burdened with the cost of Ms. Song's travel expenses in the period between 8 December 2005 and 12 February 2006, expenses totalling \$26,429.38 (Tab C, page 6/8). The disbursements claim is reduced accordingly.

[113] Research into the state of Korean law on use of seat belts added no value, in my view, to the processing or settlement of the plaintiffs' claims in this court. The sum of \$1,236.00 (Tab C, page 8/8) will be deducted from the disbursements claim.

[114] The defendants contest the claim for \$21,290.50 for photocopies (Tab C, page 8/8). The defendants concede that the plaintiffs are entitled to a reasonable amount for photocopies but assert that the "amount claimed is so excessive to be truly suspect". While I don't find the claim to be suspect, I am troubled that it has not been explained and shown to be reasonable. Accordingly, that claim shall be reduced by \$5,000.00.

[115] By my calculations, the reductions to the disbursements claim total \$102,838.15.

Disposition

[116] In all of the circumstances of this case, including the need to work with the limitations inherent in fixing costs following a settlement that the court knows so little about, costs are fixed and the defendants are hereby ordered to pay the plaintiffs the sum of \$300,000.00 for fee items plus disbursements totalling \$313,825.48 plus applicable GST.

[117] The Trial Record will be endorsed accordingly and the parties shall file Minutes of Settlement within 2 weeks addressing the disposition of the action, presumably the action will be dismissed on consent.

Moore J.

DATE: 06 May 2008

Appendix

Summary of Experts' Evidence for the jury:

Charles Tator is an experienced and, as his curriculum vitae demonstrates, an accomplished neurosurgeon.

He described having met Eugene and examining her on 27 Jan 06 and having had a telephone conversation with her on 21 December 07.

In addition, he reviewed films, performed a physical examination, reviewed hospital records.

He began with a description of the anatomy of the spinal cord and brain. He used a skeleton model and illustrations for that purpose.

He told you that in addition to injury to the brain and to other injuries, she suffered injuries to two separate parts of her spinal column and cord.

Each nerve running off the spinal cord has a different function. So, for example, numbness felt in the hands is really coming from the area of injury in the neck.

He described the difference between incomplete and complete injuries to the spinal cord. He said that there are millions of nerve fibers in the spinal cord. A complete injury means that there is no function below the level of injury whereas an incomplete injury permits of some feeling of the skin or movement of muscles controlled by the nerves exiting the spinal cord below the level of injury.

In this case, there was injury at two levels, in the neck [less severe] and in the thoracic spine [more severe].

He showed you white colors seen in the MRI image within the spinal canal which indicate bleeding from the spinal cord at the level of T 3-4. This image was taken four days after the injury and it accounts for the fact that she was not able to move her legs because of the injury.

He then took you to images taken on 23 December 2002 [5 months later] to see what had happened in the interval. At that time, a hole in the spinal cord is clearly seen. He described that as a major lesion indicative of a loss of tissue in the segment shown. This accounted for about 75% of the tissue that would normally appear in that area of the spinal cord.

This imaging amounts to the most accurate test that doctors have in assessing injury to the spinal cord, he said.

The injury to the plaintiff's cervical spine is much less severe but because it affects movement and coordination of the arms and the hands its results are significant.

Assisted by reference to various illustrations, he showed you the location and the nature of the fractures to the cervical spine demonstrated on CT scanning. He said that he feels that the illustrations are very accurate.

Also shown were illustrations depicting the thoracic spine fractures and something he described as a Syrinx or a cavity within the spinal cord in the thoracic area.

Physical examination

He described the history that he took on 27 January 2006, about 3 1/2 years after the injury.

On examination

He noticed a scar at the base of the front of her neck from the tracheostomy, an opening of the neck into the windpipe to assist breathing.

He noticed a slight drooping on the left side of Eugene's face and a brief test of mental and cognitive functioning indicated that she had a poor short-term memory. She could not remember three items for five minutes, for example.

He noted weakness in the left arm [4+] which is a slight weakness and particularly involving the small muscles of the hand. Her right arm was slightly stronger but still weak. Rapid fine finger movements were impaired for strength and coordination.

He noted variable strengths in the legs. There was zero strength for some muscles; for example, she was unable to move her feet upwards. Muscles higher up in the legs were as strong as grade four of five and therefore she was able to walk. In general, her grade level of muscle strength varied between two and four.

He noticed abnormal reflexes, leg shaking [clonus] and left ankle contracture.

There was major impairment in the sensory examination. For example, he noted pinprick impairment in the hands and legs.

He found muscle wasting. This results from the loss of nerve supply and it produces shrinkage and pinprick impairments. In the hands, there was wasting because of damage to nerve cells in the neck.

You will recall that the doctor showed you his own hands and described the muscle "belly" wasting on the top of the plaintiff's hands between the thumb and first finger.

Oxygen saturation

Another manifestation of injury to the plaintiff's brain involves oxygen saturation. To account for memory and other cognitive deficits, he said that he looked to three distinct factors.

Firstly, the injury to the brain at the time of the accident was significant. She was confused at the scene and to an extent, the problems that she has complained of with memory and concentration may find their root in the concussive injury she suffered.

The head injury alone however does not explain the profound cognitive deficits that she has.

The second cause for ongoing complaints has to do with the internal injuries to the spleen and kidney area. Those led to extensive bleeding which filled the abdominal cavity. Her blood pressure decreased and she went into shock. The level of oxygen saturation in the blood dropped.

She was intubated and her blood levels monitored. She had multiple transfusions, approximately 30 units of blood and blood components. She was critically ill and it was impossible to keep up with the profuse bleeding from her spleen.

As blood pressure kept falling, she required an immediate operation to remove the blood and the spleen from her abdomen.

There was also damage to her lungs detected bilaterally and tubes were inserted to increase oxygen flow into the lungs.

Normal oxygen levels range between 85 and 100. Hers were much lower and her oxygen carrying capacity or hemoglobin was only about one half of normal [in women it should be 140 but hers was in the 60s].

Therefore the effect of the low blood pressure etc. accounts for cognitive deficits. He described that as hypoxia or low oxygen levels reaching the brain.

Depression

The third contributor to cognitive impairments is depression. She was naturally depressed because she was acutely aware of her disabilities and limitations.

Syringomelia

The syrinx or cavity in the spinal cord was described earlier. Complaints of numbness and tingling getting worse over time are a worry to doctors because they think of some of the multitude of complications of spinal cord injuries that can arise. These complaints may be a sign of progressive enlargement of the cavity. That is potentially treatable and can be detected by serial examinations and serial MRI images.

Attendant care needs

Because this plaintiff has multiple disabilities due to three major injuries to her neck, thoracic region and brain and because it is unusual to have three anatomical regions damaged in the nervous system, he feels that she does need almost constant attendant care.

For one thing, she has memory difficulties including difficulty in recognition of danger. She would require someone to be with her almost always because of the cognitive, cerebral factors.

The fact of the existence of two other additional levels of injury means that virtually every activity of daily living is impaired significantly. For example, even simple acts like taking off boots are impaired because she does not have coordination and strength sufficient to do that and needs help, he stated.

Although she can walk and stand to some degree, because of balance and weakness problems and perhaps because of judgment problems as well, she is at risk.

She recently fell and fractured her ankle and that he said is "a wonderful example of why she needs people with her".

He has read the report of Ms. Barbara Baptiste, who he knows and respects, and he agrees that the plaintiff needs virtually full-time attendant care.

He has read the report of Dr. Oshidari but does not agree with it. He feels that that doctor did not pay sufficient attention to the fact that Eugene presents with an additional injury to her cervical spine and associated impaired use of her upper limbs.

His report relates to someone with thoracic spine injury but not to someone with the additional upper spine injury.

Cross-examination

He confirms that he had recommended in his reports in 2006 and again in 2008 that additional MRI studies be undertaken to see if there is change in the spinal cavity. He is not aware that any additional MRIs have been done since 2002.

Paraplegia

Paraplegia describes the lack of voluntary motion of the lower extremities and can include urinary and sphincter impairments.

This plaintiff does not have total loss of bladder and bowel function although it is impaired. She does not have total loss of the use of her limbs although they are impaired. She therefore does not have complete paraplegia.

He has recommended that neuro-psychiatric testing be done.

He knows Dr. Oshidari personally. He describes him as being well experienced although probably possessed of only one half of the experience that Dr. Tator says that he himself has.

He is not aware of the plaintiff's success at University. He did not apparently ask about her academic record. He did however know that she had returned to University. She told him that her intellectual capacity was about 45% of normal.

Dr Henry Berry – neurologist

This doctor is a neurologist and a specialist in internal medicine with training in [and a university degree in] psychiatry.

He explained that neurology involves the diagnosis and treatment of diseases of the nervous system. Internal medicine deals with internal organ systems.

He told you that he met, in private and without an interpreter, with the plaintiff and later met, with an interpreter, with her parents.

He described how he prefers interviewing a person in English if that person understands some English. In that way, he can better read the emotions of the person, see hesitations and intensity etc..

Records review

He told you about the information he read and considered during his review of available hospital and medical records

History from plaintiff

Her last memory and length of amnesia can be an important yardstick to the degree of injury. Eugene recalled the driver swerving and people in the car screaming and then had 15 days of amnesia.

She was in hospital for two months and then in a rehabilitation facility for three months. She then spent about a year in hospital in Korea.

The history records that she returned to school in September 2005 and took about one half of the normal course load. She did "not bad" but had to study much more.

Her complaints included: can't walk, sensations diminished, cannot feel hot and cold, can feel bowel full but cannot get it out, has bladder sensation but is nervous always about possible accidental discharge. She wets herself a few times and usually at home. Her left shoulder movement is diminished. She is seeing three doctors including a psychologist. She takes sleep medication, Tylenol and suppositories.

On functional examination, this doctor records that she lost 7 kg, her sleep is poor, she is thinking all the time, occasionally has dreams, sometimes of accidents, she cries often, feels faint at times, has headaches on most days and that she described as being bad, she wears contact lenses and complained of vision problems.

She said that she cannot speak loudly, has difficulty with some names, finds it hard to choose words, cannot swallow big things, has neck pain, has symptoms that the doctor described as anxiety attacks, once weekly, and has pain in her low back, pelvic, right leg and neck areas.

In addition, she has memory problems and decreased concentration. She forgets what she has to do.

Her past health was normal.

Her personal history he records as including that she is living with her parents, she described her typical day, said that she would go to a restaurant with friends on occasion and that she has no boyfriend.

This doctor found the plaintiff to be alert, cooperative, although notably quiet, and that she spoke in a low voice. She did not speak spontaneously; she smiled occasionally and answered slowly.

She admitted to being sad and often angry. She said she had no one to talk to and did not like to talk in any event.

Physical examination

He performed a physical examination of the plaintiff while she was seated in her wheelchair. He noted weakness in the arms and shoulder and legs and hip area. These were typical findings for her known injuries he said.

All reflexes were absent in the upper limbs which means damage to the spinal cord or to the brachial plexus. The findings were typical of a high thoracic paraplegia with some minimal functioning.

History from parents

Through an interpreter, the parents spoke of dizziness; that the plaintiff often has to lie down, and she complains of headaches, tingling in both arms down to the fingers and she complains of decreased memory. Mother would be with plaintiff while studying. The plaintiff would get angry if interrupted. She spent two to three times more in studying than she had done before.

The plaintiff was described as being afraid of being alone. Her speech was slow and she displayed word finding difficulty.

Telephone interview, 7 January 2008

By this time, the plaintiff had graduated from university (in August 2007) with above-average grades, with help from fellow students etc.

She began working in October 2006 at KAL. This was full-time work but she found it hard sitting for eight hours.

Eugene's ongoing symptoms were much the same. She complained that the numbness in her fingers was worsening. She said that she feels sad and that sometimes wants to die but "no can't do like that". She said that she cannot talk about her emotions.

Independent living

She felt that she couldn't live independently on her own because she needs help during the day. She cannot wash her legs, bottom, back. She needs help dressing and with toilet. She said that she'd needs help with work and transferring to the toilet as well. She added that she cannot go out without help. She cannot fully wash herself, dress herself or cook for herself.

She was not sure about the future and said the work was "getting hard" she added that she makes mistakes at work and feels badly about that.

Conclusions and diagnosis

He feels that she had a spinal cord injury involving a high level paraplegia with trunkal involvement together with a cervical injury producing weakness, clumsiness and loss of sensation and reflexes in the upper limbs. She also presented with a syrinx [cavity] in the spinal cord and said that this condition tends to progress. He added that she has permanent brain damage.

Brain injury diagnosis

He makes this diagnosis because of the extensive [15 days] amnesia together with the nature of her other symptoms and her depression [evident from her crying, that she wants to die but can't.]

He described the depression as a "reactive depression" in the sense that she is reacting to the fact that she is a paraplegic with associated limitations in functioning.

Prognosis

She has permanent injuries and we are now six years after the accident. Her physical, neurological condition is stable but the complaints of increasing numbness in her hands are a concern. She is at risk of progression from the Syrinx and at risk for the usual complications of paraplegia including deep vein thrombosis, pressure sores etc..

Dr. Resnik

Dr. Resnik calls the plaintiff's condition a major depression that is treatable with aggressive treatment to help resolve it.

Dr. Berry disagrees because it is a reactive depression and the patient's paraplegia is going to persist.

He also disagrees with Dr. Resnik's diagnosis of no brain injury because there is no doubt but that she has a post traumatic amnesia of two weeks. To a neurologist, a severe accident with many broken bones everywhere and post dramatic amnesia equals permanent brain damage.

Cross exam

He agrees that he takes a history because he wants to be certain of what the plaintiff was like before the accident in comparison to how she presents afterwards. In that regard, the parents can confirm or validate the history taken from the patient.

This doctor's report says that the parents were working prior to the accident but apparently they were not.

GCS

This is a rough estimation of how bad a head injury has been and used for purposes of emergency treatment or assessment. Eugene's reading was 15 which generally indicates that the plaintiff is alert and attentive and generally indicates no major brain damage. He added, however, that there can be problems to getting an accurate reading arising from a language barrier.

The GCS would have dropped, if it had been measured, at hospital as she began to lose consciousness. He estimates that it would have gone down from 15 to eight or nine or even seven. This means that she was subject to anoxic brain damage because of blood loss.

Her GCS rose to 13 eight days after the accident.

Amnesia

He agrees that post dramatic amnesia can be affected by pain medication in the first 10 days following an accident. He disagrees however with an opinion that the whole of the first two weeks of amnesia is due to medication, as is apparently suggested by Dr. Resnik.

CT scan-normal

Exhibit 2 at page 162 is the report of a CT scan taken on the day of admission to hospital. It records no brain abnormality and is a normal CT scan. This doctor is not aware of any subsequent CT or MRI scans having been done on the plaintiff's brain.

He volunteered that CT scans are often normal and falsely negative in people who have suffered brain damage.

Limbs

He records nothing about swelling of the plaintiff's lower legs on examination. He described the concept of spasticity of lower limbs being demonstrated by an increase in reflexes or stiff muscles or increased tone.

He noted no tremors are shaking of the plaintiff's lower limbs.

Advice

He was aware that the plaintiff had been receiving advice from a psychologist or perhaps a neuropsychologist and felt that she should continue to listen to her doctors and talk to the psychologist.

He understood that she was on medication when he saw her in February of 06 but makes no note of anxiety medication or antidepressants in his report written following the phone call in 2008

Studies

He was aware that she attended university but he did not receive information about her grade point average. She said that she achieved "above-average grades".

Car driving - dizziness

He was aware that she was driving a car and that she complained of dizziness but he felt that the dizziness was related to postural change and did not occur when she sat in a static position as in driving a car.

Parents not work

He knows that the parents help her but agrees that she's the only one who works outside of the family home.

Dr. Hal Scher – neuropsychologist

This doctor is a neuropsychologist. He told you that neuro-psychology is an applied science aimed at understanding how acquired brain injuries affect a person's ability to think, to regulate emotion and to function.

He admitted that he is not a medical doctor and cannot prescribe medications.

He assessed the plaintiff on 3 February 2006 and spoke to her by telephone on 19 December 2007.

A neuropsychological assessment begins with a review of medical documentation. It involves an interview of the patient for his or her point of view and it involves the application of testing involving standard tests as an important part of the process.

In an ideal world, a baseline of scores would be available for the patient but usually that is not the case and so you have to estimate functioning by comparing scores to those of healthy people and by making an educated guess on functioning of a person as compared to what would be considered normal.

Observations

The plaintiff attended and was interviewed, largely through an interpreter. The parents were interviewed the same day.

In terms of behavioral observations, he was looking for signs of anxiety, nervousness and depression. The plaintiff was obviously depressed, she responded very slowly to questions, her emotions appeared flat and the content was also flat.

Using an interpreter is not ideal but he felt that it was a valid way to go.

History

She said that she was generally doing well before the accident. She said she was at university and that although it was not quite as interesting as she had hoped it would be, she intended to continue with it but for the accident.

She seemed clearly uncomfortable at times. She said she thought her mental ability was not normal, as compared to before the accident. She spoke of difficulty maintaining information and in concentrating. She did not report that she could not function at school at all, just that it was much harder for her.

The parents indicated that she would forget appointments and forget to check her answering machine.

The parents were still together. She described her father as an "executive" and said that her mother ran a clothing store.

He recorded that both parents were university educated.

Private meeting

At one point, the interpreter left. The plaintiff then acknowledged feeling depressed. She said that she did not want to live. She said everything was dark and there was no light at the end of the tunnel. She described that other people found it difficult to communicate or understand her situation and the wheelchair.

She had been popular before the accident. Now she lacked independence and needed assistance even to use the washroom.

She completed her university program but felt that she cannot use her mental powers in a normal fashion.

She complained of pain in her back.

Tests administered

A series of tests were administered including a manual performance test administered by an assistant in a quiet room free of distractions to permit of comparisons to standard norms. The plaintiff was alone in the room with an interpreter and the doctor's assistant.

She appeared fatigued and depressed and she responded very slowly. Nevertheless the validity or reliability of the testing was acceptable.

Conclusions

There were a number of factors affecting her ability to concentrate and to work at a normal pace. The degree of impairment was greater than would be expected from a brain injury alone. There were other contributing factors such as sleep disturbances and pain as well as, likely, a hypoxic factor. Nevertheless, the psychological testing afforded the best available estimate of functioning under controlled circumstances.

In the view of this doctor, the plaintiff would have an impaired ability to function, very likely, in the real world.

Telephone conversation-19 December 2007

There was an interpreter with the plaintiff on the other end of the line. Updated information was received including that she had completed her degree. She did not recall using a tutor but did have helpers for washroom needs and physical demands at University. She reported impairment continuing in her cognitive functioning. For example, her ability to recall information was "a lot less" and she said that it took her many times more effort to study.

By that time, she had been an intern at KAL and was working as a regular employee doing a clerical function. She reported making a lot of errors at work.

She was still struggling with low mood and reported symptoms of depression such as:

- passive death wishes
- feeling of hopelessness regarding expectations of happiness in the future
- many times crying in the washroom
- stressed feeling most of the time
- fatigue problems, always in a state of exhaustion
- feeling victimized; and
- she could not do all of the duties of her job and jobs that coworkers did, due to physical barriers and to her inability to travel etc.

Final summary

Concentration and memory problems continue. He concludes that she suffered brain injury, chronic pain, anxiety, depression and sleep problems.

Progress made

Nevertheless, she had made progress because she achieved a university degree and was productively occupied through work. But her symptoms of clinical depression continued. This was no longer a major depression with suicidal risk, as had been the case in 2006.

Dr. Resnik

Dr. Resnik diagnosed no post-concussion syndrome but this doctor felt that she did meet the criteria for that diagnosis. He feels there is a significant impairment regarding social and occupational functioning.

Cross-examination

There was discussion about whether the plaintiff thought and wrote in English or Korean.

The interview and testing process was very time consuming. The interview began at about nine o'clock in the morning and the testing began just before 11 o'clock in the morning on 6 February 2006 and continued through until 5:38 pm in the afternoon. The suggestion was that that is a very long time for a person presenting with a headache and who had not slept well to persist.

IQ

The testing revealed that she had TONI-2 results in the 13th percentile of healthy North American English speaking people. That translates into an IQ in the low 80s or below average. This does not compare well to university trained people. This doctor knew that the plaintiff asserted that she had above-average grades, was an A student in high school and was in university before the accident.

McLeish letter

He was provided with source documentation and a letter from Mr. McLeish

The McLeish letter alerted the reader to a potential impact on education and on employment. As to the latter, Mr. McLeish wrote:

“In terms of part time employment during school breaks Ju-Kyung tutored younger students in mathematics. She has also worked in restaurants to help pay her way through university. She has not worked in a restaurant or any other occupation since being injured. Her prospects look very bleak in Seoul as there is a cultural prejudice against people with disabilities, inaccessible infrastructure and inaccessible transportation system.

Although the letter refers to occupational therapy and physiotherapy reports from Korea, the witness does not recall reading any of those nor anything written other than in English.

He does not recall having seen a university transcript [Exhibit 9] until last week. In referring to exhibit number nine, the number of credits after the accident is similar to the

number of credits before the accident. He agrees that the information given to him that she had returned to school and taken a one half course load appears to be in-accurate.

Student grades and grade point averages were reviewed. He was not aware that she had received a 4.5 GPA after the accident nor that her overall GPA throughout University was 3.0 or 84% out of 100%.

The plaintiff told the doctor that she was hoping to get a job as a food stylist and was considering taking a master's degree.

Ideally he would like to have a baseline for anyone he tested and he knows that a patient's report is often not 100% accurate. This is one of the reasons why he compares results to norms.

There is no question that academic performance is a valuable piece of information. The plaintiff said that she was an A student and never failed courses. The parents said that she was mentally faster and quite precocious as a child.

Was it surprising then when he saw Exhibit 9 indicating low average grades in university before the accident and two failures? So what she reported was not truthful? Yes it was not accurate, he admitted.

The interview was a mixture of English and translated communications. His report indicated that the plaintiff's English was reasonably good. He also records that the interpreter said that her Korean speech seemed normal.

He reported that she appeared to respond in a slow but succinct fashion and there were no comprehension difficulties that were obvious. There was no sign of formal thought disorder and he added that thought content appeared to be colored by depression.

Parents

It is normal to inquire into the employment and education background of parents. The plaintiff said that her father is an executive and that her mother runs a clothing store [both in the present tense] and she said that both parents were university educated.

The education of the plaintiff's parents is a factor leading to an increased likelihood that she would be of above average intellectual ability.

She told this doctor that she spent most of her time studying, even with a 50% caseload:

CT results

He was aware that the CT scan showed normal results for the brain but added that this is often the case in mild to moderate brain injury cases. He agreed however that there was no sign of intracranial bleeding and that this injury is "a concussion".

Driving

The history given by the plaintiff is the she said she is very fearful of driving, indeed too fearful to drive. At the time of February 2006 interview, he was not aware that one month later she would buy a vehicle and that she has been continually driving it for about two years and driving home from work for the past 16 months in rush hour traffic for an hour to an hour and a half. She did tell him in the subsequent telephone conversation that she had returned to driving, however.

Date of report

The interview and the date of the report are eight months apart. His only explanation is that he had taken on too much work.

Dr. David Ditor, PhD.

He is a doctor of kinesiology which, he told you, involves the study of human movement including physiology and anatomy. He teaches and conducts research into the secondary health complications arising from spinal cord injuries with an emphasis upon cardiovascular elements.

In cross-examination upon his qualifications, he confirmed that he is not a medical doctor. He spoke of his work both during and after his doctoral studies including cardiovascular investigations of the effect of exercise. He described physiology as the way by which the human body works.

He met with Eugene, her parents and an interpreter in January of 2006 and then spoke to her by telephone in late December or early January just passed.

Using the skeletal model and exhibits 21 & 22 he discussed the anatomical relationship between the spine and nerve roots.

He said that it is important not to confuse the vertebral column, the bony components, with the spinal cord inside.

He explained that the spinal cord is a neural structure. It has a thickness of about 2 cm or, as he put it, of your pinky finger. It is a soft structure with a consistency like toothpaste.

Some people refer to the spinal cord as a telephone wire but he prefers the analogy of a superhighway with lanes going up and down between the brain and the skin, muscles and internal organs.

Spinal cord injury at any given segment will reduce the information traveling between the brain and muscles, organs or skin.

He explained the difference between complete and incomplete injury and said that spinal cord injury does not involve a perfect cut across the cord. There can be spared tissue, usually at the other edge of the cord. Incomplete injury can, however, still have very broad consequences.

Testing

He tested Eugene for strength in both the upper and lower limbs and found that both were impaired. She has compromised motor function in her hands and arms. As well, she has greater motor impairments in her legs.

He explained that when muscle becomes paralyzed or partially paralyzed, both of the quantity [or mass] and the quality of the muscle can decrease. This produces deficits in both strength and endurance.

He described how the muscle becomes shortened because of lack of structure of the muscle and a person can lose range of motion.

He also described that muscle spasticity can happen and can produce random, involuntary movement or spasm.

He assessed Eugene's trunk muscles and found impairments which are important for her mobility, ability to reach, turn and lean back in a slow controlled movement. These muscles are impaired also in terms of strength and fatigue ability.

Other findings included pain, complaints of daily headache and numbness and tingling. Some of the plaintiff's described complaints got worse over the actual between his initial interview and his subsequent telephone conversation.

He explained that spinal cord injury can produce neuropathic or spontaneous pain, aberrant pain he called it, with no physical cause and that pain is usually felt below the

level of injury. For that pain, he said that there is very little effective treatment, unfortunately.

Eugene reported problems holding even a frying pan. She cannot hold a carton of milk and pour it for herself.

He understands that she works handling paperwork and ticket sales but that she requires assistance from her coworkers.

The new apartment is more accessible than the old one but it is still problematic and she is at risk of falling during transfers.

Spinal cord injuries produce a constellation of symptoms that can contribute to falls. He explained one of the difficulties of abdomen muscle weakness as producing an inability to resist blood rushing to the feet; he called this orthostatic intolerance or dizziness or lightheadedness on standing.

Eugene can write and prepare e-mails but will have difficulty working at such tasks over a long period of time.

She can breathe in and out but has difficulty with forceful exhaling such as when coughing.

She can feed herself but cannot prepare meals.

She can operate her manual wheelchair on hardwood floors in her apartment but would be challenged on carpeted surfaces, hilly surfaces or uneven ground.

She can dress her upper body but not her lower body.

She is capable of many aspects of her own grooming but has difficulty lifting her left arm up high.

She needs assistance with toilet and with most aspects of homemaking.

She has difficulty with both level transfers and uneven transfers. The former involves moving from the wheelchair to a surface level with it [plus or minus 2 inches].

She cannot use public transit.

She can engage in sexual activities and will likely be fertile but there are issues associated with pregnancy such as increased risk of pressure sores and issues at childbirth. Her babies may have lower birth weight.

Aging process

The constellation of symptoms and complaints tends to get worse as a person ages. Skin loses elasticity and mass and so her skin will be more susceptible to break down and pressure sores.

She has difficulties with constipation and that will continue. It may get worse. Digital stimulation of the anus can, over time, irritate the rectum.

She is at risk of overuse injuries to her musculo-skeletal system and those are cumulative over a lifetime.

He expects that she will be prone to complications of the nervous system such as carpal tunnel syndrome. As she ages, her abdominal strength will continue to be a problem and so will be coughing etc. also.

She will be at increased risk of developing cardiovascular disease.

Equipment recommendations

- Body weight supported treadmill
- Functional Electrically Stimulated Exercises
- Weight Resistance and Aerobic Training

He described treadmill equipment with large arms extending over the person who is suspended in a harness from them. The device allows a person to increase the time of exercise.

For safe operation, two therapists would assist the person to move her legs and a third person would stand behind to help shift body weight from side to side.

He recommends this equipment because it helps to increase ambulation and standing tolerance. Eugene would likely enjoy increased functioning of her lower legs and increased muscle mass, strength and endurance.

Increasing muscles in the buttocks would also occur and those larger muscles would protect or cushion Eugene's skin helping to protect it against pressure sores.

As well, he explained, larger muscles allow the body to store glucose and make the person less prone to developing diabetes.

Also, this exercise will tend to decrease spasms in the lower limbs and help to increase cardiovascular conditioning and help to decrease depression.

He spoke of exercising aided by electrical stimulation. He explained that electrodes can be applied to arms and legs and can be fired so as to cause involuntary contraction of muscles. Any contraction of muscles is good and leads toward increased muscle mass in the limbs.

He spoke of resistance training as an aid to increasing upper arm strength. Weight training exercise is important to both strength and endurance and will help Eugene with her transfers and allow her to lift somewhat heavier objects.

Cost of equipment

At a minimum, she should exercise on the treadmill three times per week and ideally five times per week for 1 1/2 hours each session. The treadmill will cost about \$40,000 Canadian plus tax, shipping and brokerage fees.

The resistance training equipment that he recommends would be wheelchair accessible [so no transfers are necessary] and this equipment will cost about \$13,000 US plus shipping.

Cross-examination

He interviewed Eugene in January of 2006 and he authored two reports, one dated in July 2006 and the other dated in January of 2008.

He did not recommend a power assisted wheelchair for Eugene in either report. These chairs are bigger and a "tighter fit" in a home setting. As well, he feels it is important for Eugene to maintain as much function as possible by using her arms and hands to move herself about.

He did not recommend a vehicle modified to accept a wheelchair. He pointed out that modifications can be very expensive and may take money away from other things that she might need.

He agrees that the exercise training equipment that he has recommended is "state-of-the-art". So far as he is aware, Eugene has not purchased any of the equipment that he has recommended.

He agrees that his report recommends three therapists, three times per week, indefinitely. In his evidence in chief he spoke of exercising up to five times per week. He said that would likely show more benefit but agreed that he had not recommended five sessions per week in his reports.

Cathy Butler

She is a physiotherapist and the director of clinical programs at Austin Rehabilitation (aka FIT) since 1994.

Disability management is a component of what her clinics do and she described intervening at work site and at home site to try to assist the patient to match abilities with needs and with accommodations that may be required.

Training in physiotherapy includes range of motion, strength and developing a sense of medical and physical issues overall.

Eugene was referred for assessment by Mr. McLeish in January 2006. Ms. Butler reviewed the available medical information and took a social profile from Eugene by asking questions about lifestyle before and after the accident.

For example, she learned the Eugene had been living alone, attending school and was independent in all respects before the accident. After the accident, however, she moved back in with her family into a small apartment where she needed help with many activities. She was independent in cutting food and eating but not with general housekeeping. She had an assistant at school although she was able to take regular exams and her own notes. Eugene said that she did not believe that she could get a job in food and nutrition although she had no career plan established otherwise.

Eugene advised that she did not go out with friends regularly because of accessibility problems and because she was aware of people staring at her in public and that bothers her.

Eugene listed her ongoing complaints including headaches daily and increasing, dizziness, left-sided neck pain, tingling and numbness in the arms and hands, restricted arm movement, pain on reaching upwards with the left hand, difficulty reaching around her, pain in the wrists on use, pain in the left chest wall and pain in the mid-and low back. She also complained of left hip and groin pain, difficulty rolling over in bed, spasms, tightness in her legs, difficulty sensing hot and cold and stiffness in the ankle and knees.

Sleeping

Eugene advised that she had difficulty sleeping due to thinking about the accident and other intrusive thoughts. She also complained of mood changes and of feeling depressed and isolated.

ROM study

She described conducting a study to determine how much Eugene could move all of her various body segments. This addressed both motion abilities and limitations.

Activity tolerance

She conducted tests to determine Eugene's ability to do various things over a period of time. Eugene had good tolerance working at a sedentary level. She had poor sitting balance and standing balance.

Effort testing

She conducted maximum voluntary effort testing looking for consistency and test performance overall and at the amount of variability of results as indicative of variability of effort.

She performed grip strength testing and pinch [of fingers] testing. Eugene was weak in the result. There were objective signs of effort [fingers blanched].

Mih Wa Choi OT

Ms. Choi is a graduate with a bachelor of health sciences and occupational therapy degree from Yon Sei University in Seoul, Korea. She has been a registered occupational therapist in Korea since 1997.

She worked as an occupational therapist in Korea for 2 1/2 years, approximately 6 months at a community center where she worked with children with disabilities and for about a year and a half at a general hospital in Bussan where her work included adult patients with disabilities.

She came to Canada in 2000 to develop her career as an occupational therapist. She attended for one semester at McMaster University and then successfully wrote her Canadian license exam.

She has been qualified as an occupational therapist in Canada since 2002. In Canada, she has worked as a contract employee for assessment companies doing in-home assessments, attendant care and worksite assessments for patients involved in motor vehicle accidents.

She moved in November of 2007 to Arizona where she will soon be married. She is not working at the moment but she is registered in the United States as an occupational therapist and will be looking for work there, upon her return from Korea. She wrote and passed her United States qualification exam in 2006.

She is a member of the Korean Association of occupational therapists and the Canadian Association.

In cross-examination, she described her studies toward qualifications. For instance, she clarified that her semester at McMaster University was an entry-level study program and did not lead to a Masters degree.

She does not know how many spinal cord injured patients she has worked on.

Much of her work in Canada has been in connection with capacity assessments. She estimates that about 80% of her work has been on assessments rather than on patient treatment.

She was qualified on the basis of her expertise, training and experience as an occupational therapist with both Canadian and Korean experience.

She was first contacted by an occupational therapist in Korea who had been approached by Ms. Baptiste to find a Korean speaking and experienced occupational therapist to work with her.

She traveled with Ms. Baptiste and Mr. Baum to Korea and met there with Eugene. Ms. Baptiste was a life-care planner and Mr. Baum was in charge of house modifications.

She met with Eugene's family and did a home assessment and visited with treating doctors and university staff.

The home assessment was an occupational therapy functional assessment and it was performed on October 25, 26 and 27 of 2005.

She observed how Eugene moved, the strength in her extremities, her activities of daily living at home and in the community and how she managed overall and with the help of her parents.

She formed an opinion that Eugene required attendant care on a 24 hour per day basis because the strength and range of movement in her upper extremities was impaired and she was also at risk of falling due to balance and strength issues. She was concerned that Eugene may not react appropriately to emergency situations. Although Eugene was capable of self propulsion in her wheelchair over short times and distances, in Korea the terrain was hilly, bumpy and uneven and Eugene needs help there and to transfer from her wheelchair to a car.

She attended at the Seoul National University Hospital and at Jai Sang hospital.

She met with Eugene's podiatrist, neuropsychiatrist, occupational therapist and the manager of rehabilitation medicine. She interpreted for Ms. Baptiste and made sure that cost figures she was shown were accurate.

The manager of rehabilitation medicine showed her a book in which hourly rates for doctors and other care givers were listed. She took notes from the book but was not allowed to copy the book itself.

The following specialties and associated costs came from her notes:

- \$25 per visit of 15 to 20 minutes for doctors including podiatrists, neurosurgeons etc.
- \$83 per visit for a neuropsychiatrist
- \$75 per hour for dentist
- \$85 per hour for an occupational therapist
- \$85 per hour for it physiotherapist
- \$18 per session for hydrotherapy
- \$37 per session for a speech language pathologist
- \$25 per visit of under 30 minutes for nurse
- \$30 per visit for acupuncture
- \$50 per hour for rehabilitation support worker
- \$45 per hour [12 hour shift] for non-skilled personal-care
- \$2000 per month for live-in care
- \$1500 per month for live-in nanny

- \$17 per hour [first three-hour block] for babysitter and five dollars for each additional hour
- \$25 per visit for psychological testing
- \$85 per hour for vocational counseling \$85 per hour for educational counseling.

She said that she accompanied Mr. Baum on visits to doctors and to realtors in Bundang. Her questioning concluded with her statement that there is no equivalent occupation in Korea to Ms. Baptiste's occupation.

Cross-examination

From her personal records, she confirmed that one bundle of six pages of handwritten notes records information that she made during her visit to Korea as outlined in her evidence in chief. As well, she had a one-page summary list of notations. They were made exhibits.

She confirmed that when she worked as an occupational therapist in the community center in Korea, she earned \$1300 per month and when she worked in the hospital setting, she earned \$1400 per month.

Exhibits 36 and 37 prepared by the witness, the former was prepared in Korea and the latter approximately 1 week ago.

In answers to questions addressing the specifics of the costs listed in her evidence in chief, she confirmed that not all of those costs were taken from the book shown to her by the hospital administrator. Some of the costs, for example the cost of nannies, she obtained by telephone calls to agencies. Others of the costs, she obtained from the Internet.

On Exhibit 36 at page 1, there is reference to an MRI costing \$500 Canadian approximately.

On page 2, there is reference to in-house caregivers costing \$40 for a 10 to 12 hour day and a non-skilled caregiver costing \$70 per day [24 hours]. This is information obtained by telephone.

The cost of a nurse was obtained from the Internet and is recorded as \$25 per visit of 30 minutes to one hour duration.

The cost of the childcare support worker was obtained through the Internet she notes it to be \$18 per hour for the first three hours plus five dollars per hour thereafter for weekdays and \$20 for each of the first three hours plus six dollars per hour for weekends.

On page 3, she records that father is a manager of Dongkyung and is university educated; mother is the manager of women's clothing store for the past 10 years. This is information she received from Eugene.

Lower down on that page, she names an occupational therapy student beside a phone number but does not record a price for that person's services.

There is reference to a modified van which the witness says would cost \$10,000 Canadian.

There is no cost figure for the services of a personal support worker, a registered nurse, wheelchair fitting and training and/or mattress, although the headings appear.

She explained that she let Ms. Baptiste know of prices and Ms. Baptiste wrote numbers down.

There is no reference on this exhibit to the cost of doctors, more specifically to a neuropsychiatrist costing \$83 per visit.

The witness was taken however to Exhibit 18 and asked what did Dr. Ha charge as noted in that exhibit? The charge was \$40.

There is no reference in Exhibit 18 to the cost of a dentist.

Occupational therapy is not specifically referred to in Exhibit 18 but rehabilitation therapy is and it costs \$35. Hydrotherapy costs \$15. By way of explanation, she says that usually three or four therapists oversee a group of 20 to 25 patients and so each patient is not charged individually. The patient does her own exercises under the supervision of a caregiver or parent.

She concedes that Eugene has never hired a therapist on an individual basis.

Designations such as rehabilitation support worker and personal-care worker are not recognized in Korea.

There is no reference in exhibit 36 to vocational counseling or educational counseling costs.

In Exhibit 37 there is reference to a community care planner but that is taken directly from Barbara Baptiste's report. The witness explained that the role of community care planner can be assumed by an occupational therapist and says that the cost noted is reasonable. She admits however that her own salary 10 years ago as an occupational therapist equates to about eight dollars per hour. The figure quoted in Exhibit 37 is in 2005 dollars.

When you look at Exhibit 37, you see a long list of care givers and health care specialists, a good many of which you do not see listed anywhere in Exhibit 36.

Ms. Choi confirmed that the hourly rate equivalent in Canadian dollars is \$3.75/hr.

She was taken to information that counsel obtained from a Korean Ministry of Labour web site listing Korean Network for Occupation worker pay information. She agreed that OT earnings listed there for the period of 2005-2007 of about \$30,000.00 is reasonable.

In Ex 36, she shows personal caregiver rates from a private agency at \$4.00 per hour. The web site shows them at a lower amount.

Re-Examination

In Re-Exam, she said that her Korean OT colleagues earned \$30,000.00 per year for working 8-9 hour days. She was not sure what hourly rate an agency would charge them out at.

She said that a personal aid worker earning \$13,000.00 per year could be unskilled.

Why do the occupations listed in Ex. 37 not appear in Ex. 36? Many were people in fields beyond OT and in the areas of interest to Ms. Baptiste – life care planning people.

End of Expert Evidence Summary